

Paper Title:

Advances in Breast Cancer Segmentation: A Comprehensive Review

Paper Link:

https://www.acadlore.com/article/ATAIML/2024_3_2/ataiml030201

1 Summary**1.1 Motivation**

Breast cancer is one of the major causes of the higher death rate among women globally. One of the ways to lessen the death toll is earlier detection, diagnosis and treatment planning.

1.2 Contribution

This paper reviews different segmentation approaches used in breast cancer imaging, focusing on their strength and limitations for clinical usage. It evaluates each method's effectiveness, discusses the hurdles faced in resource collection for researchers and clinicians trying to understand and implement effective segmentation techniques in breast cancer imaging.

1.3 Methodology

The authors perform a comprehensive analysis of breast cancer segmentation methods, classifying them into four primary categories: traditional techniques (such as thresholding and edge detection), machine learning, deep learning (particularly U-Net and CNNs), and manual segmentation. They assess the effectiveness of each method using metrics including the Dice coefficient, F-score, IoU, and AUC, emphasizing deep learning as the most advantageous due to its superior accuracy and automation potential.

1.4 Conclusion

The study describes the effectiveness of deep learning methods, particularly U-Net and Convolutional Neural Networks (CNNs), in the segmentation of breast cancer. However, their implementation in clinical settings is obstructed by challenges, including insufficient annotated data and privacy issues.

2 Limitations**2.1 First Limitation**

Limited availability of annotated datasets reduces model generalizability. More images will help the model to learn better. However, excessive data can also cause problems.

2.2 Second Limitation

Privacy concerns slow down data sharing and large-scale model training.

3 Synthesis

The analysis underlines the robustness of deep learning techniques, mainly U-Net and CNNs, in breast cancer segmentation. However, the clinical use of these models is limited by data

shortage and privacy issues, calling for collaboration and anonymized datasets to support universal adoption.